TriageAI

A rule-based symptom triage and blood-panel screening aid

Work in progress. Prepared for an honest professional review.

We are asking for your critical judgment on safety, wording and clinical sense.

This deck is the whole review: 6 pages, about 10 minutes.

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What it is, and what it is not

The problem we are trying to help with

Many people have no easy way to judge whether a symptom needs urgent care, a clinic visit, or home care.

Many also receive blood results that are filed without anyone checking for early warning markers.

TriageAI is a first-step aid for both. It points a person towards the right level of care.

IT IS

- A screening aid
- Rule-based: every rule can be read and checked
- Designed for SMS or simple web use

IT IS NOT

- A diagnosis
- A replacement for a clinician
- A black-box AI model deciding anything

Every output carries the line: "Screening aid only, not a diagnosis. Always consult a qualified clinician."

Module 1: symptom triage

How it decides today (exactly as built)

The patient types their symptoms. The tool looks for set phrases.

URGENT: seek care within the hour

chest pain | difficulty breathing | severe bleeding | loss of consciousness | stroke symptoms | severe abdominal pain | high fever with rash

MODERATE: see a clinic in 24 to 48 hours

persistent cough | high fever | vomiting | severe headache | unexplained weight loss

ROUTINE: anything else, by default

Known limits we want you to test

It matches exact phrases only. "My chest is tight" or "I can't breathe properly" would not match, and would be rated Routine.

Only 12 phrases in total. No age, sex, pregnancy, duration or severity questions.

It always suggests one clinic. There is no real location logic yet.

The wording and the tiers have not been reviewed by a clinician.

Module 2: blood-panel screening

Compares entered values to fixed reference ranges

Marker	Range used	Flag rule
WBC (x10^9/L)	4.0 to 11.0	Outside range: flag
PSA (ng/mL)	0.0 to 4.0	Outside range: flag
CEA (ng/mL)	0.0 to 2.5	Outside range: flag
CA-125 (U/mL)	0.0 to 35.0	Outside range: flag
AFP (ng/mL)	0.0 to 10.0	Outside range: flag

Questions for you

Are these ranges appropriate, and are these the right five markers?

Single fixed ranges ignore age, sex and lab method. Is that acceptable for a screening flag, or unsafe?

Is tumour-marker flagging on its own misleading, since these markers can rise for benign reasons?

Is the output wording right: "flagged for specialist review"?

Built for one country and one language at a time

Each version is localised, not a single global app







Illustrations like these appear next to each question, so a person with low literacy can recognise the symptom.

Your call: what must change for your country or language? Which local terms for these symptoms do patients actually use?

One country, one language

Local emergency numbers and clinics

Local language and phrasing, such as pidgin where suited

Local medical guidance reviewed by clinicians who practise there

Safeguards and what we have not done

Please be as critical as you like

In place

A disclaimer on every result

The clinician makes the final judgment

No account, name or personal health data is requested

Counters start from zero and count real use only

Not yet done

No clinical validation or testing with real patients

No review of the urgent and moderate lists by a doctor or nurse

No handling of children, pregnancy or chronic conditions

No local-language or pidgin phrasing

No real clinic directory

What we are asking of you

A few lines of honest feedback is plenty

1. Is anything on pages 3 and 4 unsafe, wrong or misleading?
2. What important symptoms or red flags are missing?
3. Is the wording clear and appropriate for ordinary patients?
4. Would you consider this safe as a first-step aid, with changes, or not at all?
5. What is the single most important thing to fix first?

How to reply

A voice note, a message or a marked-up copy of this deck, whichever is easiest.

Contact: Tom Ugbodaga

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Thank you for your time and your honesty.